EMERGENCY SURGERY · MIDTERM · COMPLETE QUESTION BANK

Emergency Surgery — Midterm All Lectures Question Bank

جراحة / طب الطوارئ — منهج الميدترم — مجمّع أسئلة المادة كلها

4	11	198	59
Lectures	Past exam questions	Expected Qs	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	Expected	Cards
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المصادر:

كويز سابق (جراحة طوارئ)

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). الأسئلة المتوقعة عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 1

Acute Abdomen

البطن الحادة

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1.1 The most common site of peptic ulcer is: كويز سابق (جراحة طوارئ)

a. 1st part of duodenum
b. Lesser curvature
c. Fundus of stomach
d. Distal end of esophagus

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

1.N1 The term acute abdomen refers to: MCQ

a. Abdominal mass
b. Acute abdominal pain
c. Abdominal distension only
d. Chronic abdominal pain

1.N2 To reach a proper diagnosis of acute abdomen, all of the following are needed EXCEPT: MCQ

a. Immediate laparotomy for all patients
b. Laboratory and radiological investigations
c. Complete physical examination
d. Careful history taking

1.N3 All of the following are upper abdominal causes of acute abdomen EXCEPT: MCQ

a. Colonic diverticulitis
b. Perforated peptic ulcer
c. Acute pancreatitis
d. Acute cholecystitis

1.N4 Acute myocardial infarction may present as upper abdominal pain. TRUE / FALSE

a. True
b. False

1.N5 Which of the following is a cause of right lower abdominal pain? MCQ

a. Mesenteric adenitis
b. Colonic diverticulitis
c. Perforated peptic ulcer
d. Leaking aortic aneurysm

1.N6 Colonic diverticulitis is a cause of: MCQ

a. Upper abdominal pain
b. Right lower abdominal pain
c. Left lower abdominal pain
d. Pelvic pain only

1.N7 All of the following are medical causes of acute abdominal pain EXCEPT: MCQ

a. Familial Mediterranean fever
b. Diabetic ketoacidosis
c. Uremia
d. Acute appendicitis

1.N8 Mesenteric vascular occlusion can cause both upper and mid-abdominal pain. TRUE / FALSE

a. True **b.** False

1.N9 The most common acute abdominal surgical emergency is: MCQ

a. Acute cholecystitis **b.** Intestinal obstruction
c. Acute appendicitis **d.** Perforated peptic ulcer

1.N10 Acute appendicitis is commonest in: MCQ

a. The second and third decades **b.** The fifth decade
c. Infants **d.** The elderly

1.N11 Appendicitis is rare in infants because the appendix is: MCQ

a. Full of lymphoid tissue **b.** Wide-mouthed and well drained
c. Narrow and obstructed **d.** Retrocecal

1.N12 Obstructive acute appendicitis represents about: MCQ

a. 10% of cases **b.** All cases
c. 1/3 of cases **d.** 2/3 of cases

1.N13 The usual cause of obstruction of the appendix lumen is: MCQ

a. Faecolith **b.** Worms
c. Tumor **d.** Foreign body

1.N14 Non-obstructive appendicitis is more likely to perforate than obstructive appendicitis. TRUE / FALSE

a. True **b.** False

1.N15 An appendicular mass usually forms within: MCQ

a. 24 hours **b.** 3–5 days
c. 3 weeks **d.** 3 months

1.N16 Portal pyaemia as a complication of appendicitis means: MCQ

a. Peritonitis
b. Thrombophlebitis of the portal venous system with hepatic abscess
c. Intestinal obstruction
d. Pelvic abscess

1.N17 All of the following are possible courses of acute appendicitis EXCEPT: MCQ

a. Malignant transformation **b.** Appendicular mass
c. Gangrene **d.** Resolution

1.N18 The scoring system used in diagnosis of acute appendicitis is: MCQ

a. Child score **b.** Alvarado score (MANTRELS)
c. Glasgow score **d.** Ranson score

1.N19 **In the Alvarado score, which items score 2 points?** MCQ

a. Migration of pain and fever
b. Anorexia and nausea
c. Rebound and shift to the left
d. Tenderness in the right lower quadrant and leucocytosis

1.N20 **In acute appendicitis, vomiting always occurs before pain.** TRUE / FALSE

a. True **b.** False

1.N21 **The pain of acute appendicitis characteristically:** MCQ

a. Is in the left iliac fossa
b. Starts around the umbilicus then shifts to the right lower abdomen
c. Starts in the right iliac fossa then becomes generalized
d. Starts in the back

1.N22 **Absence of leucocytosis excludes acute appendicitis.** TRUE / FALSE

a. True **b.** False

1.N23 **Rovsing's sign is pain in the right iliac fossa on:** MCQ

a. Deep palpation of the left lower quadrant
b. Coughing
c. Extension of the right thigh
d. Pressing on the right hypochondrium

1.N24 **Psoas sign is elicited by:** MCQ

a. Deep palpation of the left iliac fossa
b. Flexion of the left hip
c. Extension of the right thigh with the patient lying on the left side
d. Rectal examination

1.N25 **Rigidity in a patient with appendicitis suggests:** MCQ

a. Appendicular mass **b.** Perforation
c. Mesenteric adenitis **d.** Resolution

1.N26 **Appendicitis in the elderly is more likely to perforate because of:** MCQ

a. Prominent vomiting
b. Well developed omentum
c. Little tenderness, late diagnosis and low immunity
d. Wide lumen

1.N27 **In infants and young children appendicitis is dangerous because of all EXCEPT:** MCQ

a. Greater omentum not well developed
b. Difficult examination
c. Prominent vomiting so misdiagnosed as enteritis
d. Pain displaced upwards

1.N28 **In pregnancy, appendicitis may be misdiagnosed as:** MCQ

a. Ectopic pregnancy only
b. Pyelitis
c. Cholecystitis
d. Gastroenteritis

1.N29 **Mesenteric adenitis is characterized by:** MCQ

a. Fixed tenderness
b. Tenderness that shifts with change of position, usually in children
c. Occurring only in elderly
d. High amylase

1.N30 **Meckel's diverticulitis is usually diagnosed:** MCQ

a. By CT
b. By ultrasound
c. After exploration
d. By blood tests

1.N31 **Diagnosis of acute appendicitis is essentially:** MCQ

a. Radiological
b. By laboratory tests only
c. By laparoscopy only
d. Clinical

1.N32 **Appendectomy should not be delayed especially in all of the following EXCEPT:** MCQ

a. Young healthy adults with appendicular mass
b. Pregnant
c. Elderly
d. Children

1.N33 **The treatment of appendicular mass is:** MCQ

a. Ochsner–Sherren regimen then interval appendectomy after 3 months
b. Immediate appendectomy
c. Discharge without follow-up
d. Laparotomy and drainage

1.N34 **The treatment of appendicular abscess is:** MCQ

a. Antibiotics only
b. Observation
c. Sonar-guided aspiration, surgical drainage if failed
d. Immediate appendectomy

1.N35 Calcular cholecystitis represents about: MCQ

a. 50%
b. 75%
c. 98%
d. 10%

1.N36 Acute cholecystitis is caused by: MCQ

a. Obstruction of the common bile duct
b. Obstruction of the cystic duct or neck of the gallbladder by a stone
c. Pancreatitis
d. Bacterial infection without stones

1.N37 The pain of acute cholecystitis lasts: MCQ

a. Less than 3 hours
b. More than 6 hours
c. A few seconds
d. Less than 1 hour

1.N38 Positive Murphy's sign is characteristic of: MCQ

a. Acute cholecystitis
b. Acute appendicitis
c. Diverticulitis
d. Acute pancreatitis

1.N39 In acute cholecystitis liver function tests are usually: MCQ

a. Markedly elevated
b. Show high ammonia
c. Show low albumin
d. Normal

1.N40 Ultrasound findings of acute cholecystitis include all EXCEPT: MCQ

a. Air under the diaphragm
b. Gall stones
c. Thickened wall
d. Pericholecystic fluid collection

1.N41 Early cholecystectomy is recommended within _____ of the acute attack: MCQ

a. 3 days
b. 3 weeks
c. 3 months
d. 6 weeks

1.N42 If a patient with acute cholecystitis deteriorates during conservative treatment, the treatment is: MCQ

a. Continue antibiotics
b. ERCP only
c. Surgery: cholecystectomy or cholecystostomy
d. Discharge

1.N43 Perforated peptic ulcer may be related to intake of: MCQ

a. NSAIDs or corticosteroids
b. Antacids
c. Iron
d. Antibiotics

1.N44 The ratio of duodenal to gastric perforated ulcer is: MCQ

a. 1:1
b. 6:1
c. 1:6
d. 10:1

1.N45 Board-like rigidity is characteristic of: MCQ

a. Acute pancreatitis
b. Mesenteric adenitis
c. Perforated peptic ulcer
d. Uremia

1.N46 The best investigation for perforated peptic ulcer is: MCQ

a. Barium enema
b. Abdominal ultrasound
c. Serum lipase
d. Erect chest X-ray showing air under the diaphragm

1.N47 The surgical treatment of perforated duodenal ulcer is: MCQ

a. Cholecystectomy
b. Graham's omental patch
c. Whipple operation
d. Hartmann procedure

1.N48 The aetiology of diverticular disease is thought to be: MCQ

a. Infection
b. Lack of fibre leading to chronic constipation and increased intracolonic pressure
c. Genetic only
d. High fibre diet

1.N49 The commonest site of colonic diverticulitis is: MCQ

a. Transverse colon
b. Caecum
c. Sigmoid colon
d. Rectum

1.N50 Barium enema is the investigation of choice during an acute attack of diverticulitis. TRUE / FALSE

a. True
b. False

1.N51 Diverticulitis with generalized peritonitis is treated by: MCQ

a. Exploration with Hartmann's procedure
b. U/S guided drainage
c. Graham's patch
d. Conservative treatment

1.N52 Mesenteric ischemia is a type of: MCQ

a. Simple intestinal obstruction
b. Paralytic ileus
c. Strangulation intestinal obstruction
d. Peptic ulcer

1.N53 Mesenteric arterial embolism may be caused by: MCQ

a. Peptic ulcer
b. Atrial fibrillation or mural thrombus after MI
c. Pancreatitis
d. Hypertension

1.N54 Mesenteric ischemia is characterized by: MCQ

a. Chronic pain only
b. Minimal symptoms with maximum signs
c. No pain
d. Maximum symptoms with minimal signs

1.N55 The investigation that shows the cause and status of the bowel in mesenteric ischemia is: MCQ

a. Barium meal
b. Plain X-ray
c. Ultrasound
d. CT angiography

1.N56 In mesenteric ischemia with gangrenous bowel, a second look operation is done after: MCQ

a. 6 hours
b. 24 hours
c. 1 week
d. 1 month

1.N57 Mesenteric venous thrombosis with viable bowel is treated by: MCQ

a. Bypass graft
b. Anticoagulation therapy
c. Resection
d. Embolectomy

1.N58 Plain X-ray of the abdomen in acute abdomen mainly detects: MCQ

a. Pancreatitis
b. Gallstones
c. Stones of the urinary tract
d. Ovarian torsion

1.N59 Abdominal ultrasound can diagnose all of the following EXCEPT: MCQ

a. Ruptured ovarian cyst or ovarian torsion
b. Renal stones
c. Acute calcular cholecystitis
d. Air under the diaphragm

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Acute abdomen	Acute abdominal pain needing urgent diagnosis: careful history, full exam, labs and imaging.	**Upper abdominal causes**	Perforated peptic ulcer, biliary colic/cholecystitis, acute pancreatitis, leaking AAA, MI.
RIF causes	Appendicitis, mesenteric adenitis, right ureteric colic, right ovarian cyst rupture/torsion.	**LIF cause**	Colonic diverticulitis (sigmoid), left ureteric colic.
Medical causes	DKA, MI, uremia, gastroenteritis, familial Mediterranean fever.	**Appendicitis**	Most common surgical emergency; peak 2nd–3rd decades; faecolith obstruction in 2/3.
Appendicitis course	Resolution, gangrene, perforation → peritonitis, appendicular mass (3–5 days), abscess, portal pyemia.	**Appendicitis signs**	McBurney tenderness, rebound, Rovsing's, psoas sign; Alvarado score (MANTRELS).
Appendicular mass	Ochsner–Sherren conservative regimen → interval appendectomy after 3 months.	**Acute cholecystitis**	98% calculus; RUQ pain > 6 h → right shoulder; Murphy's sign; US first.

Cholecystitis treatment	Early cholecystectomy within 3 days, or conservative then surgery after 6 weeks.
Peptic ulcer site	Most common: 1st part of the duodenum (duodenal : gastric = 6 : 1).
Mesenteric ischemia	Pain out of proportion to signs; CT angiography; urgent surgery (embolectomy / bypass / anticoagulation).
Perforated peptic ulcer	Sudden epigastric pain, board-like rigidity, air under diaphragm; Graham's omental patch.
Diverticulitis	Low-fiber diet; sigmoid; CT with contrast; NPO, IV fluids, antibiotics; Hartmann if peritonitis.

LECTURE 2

Acute Pancreatitis

التهاب البنكرياس الحاد

Part A · Past Exam Questions

أسئلة الدفعات السابقة

2.1 The most common cause of acute pancreatitis: كويز سابق (جراحة طوارئ)

a. Idiopathic
b. Alcoholism
c. Bile duct stone
d. Iatrogenic

2.2 The best treatment of acute pancreatitis is: كويز سابق (جراحة طوارئ)

a. Conservative treatment
b. CT guided aspiration
c. Laparotomy and debridement
d. None of the above

2.3 The most sensitive test for diagnosis of acute pancreatitis is: كويز سابق (جراحة طوارئ)

a. Serum calcium level
b. Urinary amylase level
c. Serum amylase level
d. Serum lipase level

2.4 As regards surgical drainage of pancreatic pseudocyst, all are correct EXCEPT: كويز سابق (جراحة طوارئ)

a. Indicated in all cases
b. Done by cystogastrostomy
c. Indicated in infection
d. Indicated if the cyst > 6 cm

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

2.N1 The pancreas is a/an: MCQ

a. Pelvic organ
b. Upper abdominal retroperitoneal organ
c. Mediastinal organ
d. Intraperitoneal organ

2.N2 The main pancreatic duct joins the common bile duct at the ampulla of Vater which opens into: MCQ

a. 3rd part of duodenum
b. 2nd part of duodenum
c. 1st part of duodenum
d. Jejunum

2.N3 The pancreas secretes daily about: MCQ

a. 100–200 mL
b. 1–2 litres
c. 5 litres
d. 500 mL

2.N4 The most common cause of acute pancreatitis is: MCQ

a. Trauma
b. Alcohol
c. Hypertriglyceridemia
d. Bile duct stones

2.N5 Excess alcohol intake causes about _____ of cases of acute pancreatitis: MCQ

a. 10%
b. 30%
c. 50%
d. 70%

2.N6 All of the following are causes of acute pancreatitis EXCEPT: MCQ

a. ERCP
b. Hypertriglyceridemia
c. Hypothyroidism
d. Hyperparathyroidism

2.N7 Acute pancreatitis can follow ERCP. TRUE / FALSE

a. True
b. False

2.N8 The pathogenesis of acute pancreatitis is: MCQ

a. Bacterial infection of the pancreas
b. Premature activation of pancreatic enzymes inside the gland leading to autodigestion
c. Pancreatic duct tumor
d. Insulin deficiency

2.N9 Inflammatory cytokines released in acute pancreatitis may lead to: MCQ

a. Hypertension
b. Polycythemia
c. Hyperkalemia
d. SIRS

2.N10 Hypocalcemia in severe acute pancreatitis is caused by: MCQ

a. Vomiting
b. Renal loss
c. Fatty acids binding calcium forming calcium soaps (fat necrosis)
d. Low vitamin D

2.N11 The pain of acute pancreatitis is: MCQ

a. Lower abdominal colicky pain
b. Epigastric pain radiating to the back, acute onset and gradually increasing
c. Pain referred to the left shoulder
d. Right iliac fossa pain

2.N12 Patients with acute pancreatitis often sit: MCQ

a. Prone
b. Supine
c. Trendelenburg
d. Leaning forward (prayer position)

2.N13 Local signs of acute pancreatitis are less impressive than expected from severity of pain because the pancreas is: MCQ

a. Retroperitoneal
b. Small
c. Intraperitoneal
d. Painless

2.N14 Cullen's sign is: MCQ

a. Bruising around the umbilicus
b. Pain in the right shoulder
c. Jaundice
d. Bruising in the loin

2.N15 Grey Turner's sign is: MCQ

a. Rigidity
b. Tenderness at McBurney's point
c. Bruising around the umbilicus
d. Bruising in the loin (flank)

2.N16 Upper abdominal swelling 2–3 weeks after an attack of acute pancreatitis suggests: MCQ

a. Pancreatic pseudocyst
b. Perforated ulcer
c. Gallbladder mucocele
d. Pancreatic cancer

2.N17 All of the following are systemic complications of acute pancreatitis EXCEPT: MCQ

a. ARDS
b. Shock
c. Paralytic ileus
d. Pancreatic abscess

2.N18 Renal failure in acute pancreatitis is due to: MCQ

a. Hypovolemia
b. Nephrotoxic enzymes
c. Stones
d. Hypercalcemia

2.N19 Tetany in acute pancreatitis is due to: MCQ

a. Hypokalemia
b. Hyponatremia
c. Hypocalcemia
d. Hyperglycemia

2.N20 Differential diagnosis of acute pancreatitis includes all EXCEPT: MCQ

a. Acute appendicitis
b. Acute cholecystitis
c. Myocardial infarction
d. Perforated peptic ulcer

2.N21 In acute pancreatitis serum amylase is usually elevated to more than: MCQ

a. 100 IU
b. 500 IU
c. 1000 IU
d. 10,000 IU

2.N22 Serum amylase is specific for acute pancreatitis. TRUE / FALSE

a. True
b. False

2.N23 The more specific enzyme for acute pancreatitis is: MCQ

a. LDH
b. Lipase
c. Amylase
d. ALT

2.N24 Arterial blood gases in acute pancreatitis are done to: MCQ

a. Measure amylase
b. Detect patients who will need mechanical ventilation
c. Detect stones
d. Diagnose pancreatitis

2.N25 **Laboratory findings in acute pancreatitis include all EXCEPT:** MCQ

a. Hypocalcemia
b. Elevated blood urea
c. Hypoglycemia
d. Hyperglycemia

2.N26 **Abdominal ultrasound in acute pancreatitis may show:** MCQ

a. Air under diaphragm
b. Gallbladder stones and dilated biliary tree
c. Kerley B lines
d. Pancreatic necrosis clearly

2.N27 **The investigation that shows pancreatic necrosis is:** MCQ

a. Plain X-ray
b. CT scan with IV contrast
c. Serum lipase
d. Ultrasound

2.N28 **MRCP is indicated in acute pancreatitis if:** MCQ

a. CBD stones are suspected
b. Always
c. Pseudocyst is present
d. Pancreatic necrosis is suspected

2.N29 **The first line treatment of acute pancreatitis is:** MCQ

a. Antibiotics only
b. ERCP in all cases
c. Conservative treatment (R regimen)
d. Urgent laparotomy

2.N30 **Pain in acute pancreatitis is relieved by pethidine combined with atropine to:** MCQ

a. Increase heart rate
b. Counteract spasm of the sphincter of Oddi
c. Reduce secretion only
d. Prevent vomiting

2.N31 **Rest of the pancreas in acute pancreatitis is achieved by:** MCQ

a. Keeping the patient NPO
b. Laxatives
c. Early ambulation
d. High-protein diet

2.N32 **Prophylactic antibiotics in acute pancreatitis are now used only in:** MCQ

a. Necrotizing pancreatitis (carbapenems preferred)
b. All patients
c. Mild pancreatitis
d. Alcoholic pancreatitis

2.N33 **An obstructing bile duct stone in acute pancreatitis is removed by:** MCQ

a. ESWL
b. Colonoscopy
c. ERCP
d. Laparotomy

2.N34 **Surgical treatment of acute pancreatitis includes all EXCEPT:** MCQ

a. Excision of necrotic tissue in severe necrotizing pancreatitis
b. Routine pancreatectomy for mild cases
c. Drainage of pancreatic abscess
d. Drainage of persistent pseudocyst

2.N35 **Pancreatic pseudocyst is lined by:** MCQ

a. Epithelium
b. Mucosa
c. Endothelium
d. Inflammatory tissue (false cyst)

2.N36 **Pancreatic pseudocyst develops after acute pancreatitis in about:** MCQ

a. 1% of cases
b. 10% of cases
c. 50% of cases
d. 90% of cases

2.N37 **Pancreatic pseudocyst is diagnosed by:** MCQ

a. CT scan
b. Barium meal
c. Plain X-ray
d. Serum amylase

2.N38 **Most pancreatic pseudocysts resolve spontaneously.** TRUE / FALSE

a. True
b. False

2.N39 **A pancreatic pseudocyst is surgically drained if it does not resolve within:** MCQ

a. 1 week
b. 2 weeks
c. 6 weeks
d. 6 months

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Pancreas	Retroperitoneal; main duct joins CBD at ampulla of Vater (2nd part of duodenum); 1–2 L juice/day.	**Causes**	Gallstones 50%, alcohol 30%, idiopathic 20%; trauma/ERCP, hyperparathyroidism, hypertriglyceridemia.
Pathogenesis	Premature enzyme activation → autodigestion → cytokines → SIRS.	**Fat necrosis**	Lipase → fatty acids bind Ca^{2+} (calcium soaps) → hypocalcemia → tetany.
Pain	Upper abdominal, radiating to the back; patient leans forward; vomiting.	**Signs**	Fever, tachycardia, shock; Cullen's (periumbilical), Grey Turner's (flank) bruising.
Amylase	> 1000 IU, raised 2–3 days; not specific (perforated ulcer < 500).	**Lipase**	More specific (and sensitive) for acute pancreatitis.

Imaging	US: gallstones; CT with IV contrast: necrosis, edema, fluid; MRCP: CBD stones.
Local complications	Pancreatic pseudocyst, pancreatic abscess, necrosis.
Surgery	Excise infected necrosis, drain abscess, drain persistent pseudocyst.

Systemic complications	Shock, ARDS, renal failure, paralytic ileus, tetany.
R regimen	Relief of pain, Replacement of fluids, Respiratory support, Rest (NPO), Resistance to infection, Removal of stones (ERCP).
Pseudocyst	Juice + exudate without epithelial lining; 2–3 weeks after attack; most resolve; drain if > 6 weeks (cystogastrostomy).

LECTURE 3

Burn

الحروق

Part A · Past Exam Questions

أسئلة الدفعات السابقة

3.1 According to the rule of nines, a 40-year-old patient who has burns over the front of chest and abdomen has about: كويز سابق (جراحة طوارئ)

a. 24% burns
b. 36% burns
c. 9% burns
d. 18% burns

3.2 The burn injury that occurs due to contact with strong acid or alkali is known as: كويز سابق (جراحة طوارئ)

a. Chemical burn
b. Inhalational burn
c. Thermal burn
d. Radiation burn

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

3.N1 By size and weight, the skin is the _____ largest organ in the body: MCQ

a. First
b. Second (after muscles)
c. Third
d. Fourth

3.N2 The epidermis is formed of: MCQ

a. Columnar epithelium
b. Stratified squamous epithelium
c. Muscle
d. Connective tissue

3.N3 Skin appendages include all EXCEPT: MCQ

a. Sweat glands
b. Sebaceous glands
c. Hair follicles
d. Lymph nodes

3.N4 Healing of second-degree burns occurs by epithelium creeping from the skin appendages. TRUE / FALSE

a. True
b. False

3.N5 All of the following are functions of the skin EXCEPT: MCQ

a. Formation of vitamin K
b. Barrier against micro-organisms
c. Prevents excessive water loss
d. Formation of vitamin D

3.N6 Scalds are caused by: MCQ

a. Electricity
b. Flame
c. Boiled liquids
d. Acids

3.N7 Flame burns are often due to: MCQ

a. Radiation
b. Hot liquids
c. Acids
d. Ignition of clothing

3.N8 Electric burns with high voltage may be fatal because: MCQ

a. They are superficial
b. They cause scalds
c. Blood and bones are good conductors
d. Skin is a good conductor

3.N9 Inhalation burns are due to exposure to: MCQ

a. Hot liquids
b. Hot gases
c. Sun
d. Acids

3.N10 The extent of burn is estimated by: MCQ

a. Parkland formula
b. Glasgow scale
c. Lund formula
d. Rule of 9

3.N11 A major burn involves more than _____ of body surface area: MCQ

a. 10%
b. 15%
c. 30%
d. 50%

3.N12 An intermediate burn in adults involves: MCQ

a. < 15%
b. 15–30%
c. 30–50%
d. > 50%

3.N13 A minor burn in children is less than: MCQ

a. 5%
b. 10%
c. 15%
d. 20%

3.N14 According to the rule of nines, each upper limb represents: MCQ

a. 1%
b. 9%
c. 18%
d. 27%

3.N15 According to the rule of nines, the perineum represents: MCQ

a. 1%
b. 9%
c. 18%
d. 4.5%

3.N16 The usual example of first-degree burn is: MCQ

a. Chemical burn
b. Scald
c. Sunburn
d. Electric burn

3.N17 Second-degree burns heal within _____ if no infection occurs: MCQ

a. 3 months
b. Never
c. 3 weeks
d. 3 days

3.N18 **Infection of a second-degree burn may change it into a full-thickness burn.** TRUE / FALSE

a. True **b.** False

3.N19 **In third-degree burns there is:** MCQ

a. Complete destruction of epidermis and dermis
b. Damage of epidermis and part of dermis
c. Erythema only
d. Damage of epidermis only

3.N20 **Inhalation injury may lead to all EXCEPT:** MCQ

a. Pneumonia **b.** Upper airway edema
c. Hypertension as the main problem **d.** Bronchospasm

3.N21 **Neurogenic shock in burns is caused by:** MCQ

a. Fluid loss **b.** Infection
c. Pain **d.** Heart failure

3.N22 **Acute renal failure in burns occurs after:** MCQ

a. Antibiotics **b.** Prolonged uncorrected hypovolemia
c. Skin grafts **d.** Excess fluids

3.N23 **Curling's ulcer is:** MCQ

a. Acute ulceration of the stomach and duodenum after burns
b. Corneal ulcer
c. Ulcer in an old burn scar
d. Pressure ulcer

3.N24 **Burn wound infection usually occurs between:** MCQ

a. Day 1–2 **b.** Days 4–7 postburn
c. 3 weeks **d.** 3 months

3.N25 **The value of systemic antibiotics in prevention of burn wound infection is doubtful.** TRUE / FALSE

a. True **b.** False

3.N26 **Constricting eschars in deep circumferential burns of limbs and chest are treated by:** MCQ

a. Urgent escharotomy **b.** Antibiotics
c. Skin graft **d.** Tracheostomy

3.N27 **Edema in burns of the face and neck may need:** MCQ

a. Urgent tracheostomy **b.** Escharotomy
c. Skin graft **d.** Fasciotomy

3.N28 **Marjolin's ulcer is:** MCQ

a. Acute gastric ulcer
b. Infected burn
c. Keloid
d. Malignant transformation in a long-standing burn scar

3.N29 **Local complications of burns include all EXCEPT:** MCQ

a. Compartment syndrome
b. Curling's ulcer
c. Contractures
d. Hypertrophic or keloid scar

3.N30 **First aid for burns includes pouring water over the burn for:** MCQ

a. 1 hour
b. 1 minute
c. 15 minutes
d. Ice-cold water for 30 minutes

3.N31 **Using ice-cold water in first aid of burns is contraindicated because it:** MCQ

a. Delays healing only
b. May induce more tissue damage
c. Causes infection
d. Is painful

3.N32 **Minor burns can be treated as outpatients.** TRUE / FALSE

a. True
b. False

3.N33 **Indications for hospital admission in burns include all EXCEPT:** MCQ

a. Chemical burns
b. Electric burns
c. Inhalation injury
d. First-degree sunburn of 5%

3.N34 **On admission of a burn patient, a Foley catheter is inserted to:** MCQ

a. Prevent infection
b. Measure CVP
c. Give drugs
d. Check urine output

3.N35 **The Parkland formula is:** MCQ

a. 2 mL/kg/% burn saline
b. 4 mL/kg/% burn lactated Ringer's per day
c. 4 mL/kg/% burn dextrose
d. 1 mL/kg/% burn plasma

3.N36 **According to Parkland formula, half of the amount is given in:** MCQ

a. The first 4 hours
b. The second day
c. The first 8 hours
d. The first 24 hours

3.N37 **In all fluid formulae, the maximum percentage of burn calculated is:** MCQ

a. 30%
b. 50%
c. 70%
d. 100%

3.N38 Oral intake in major burns is avoided during the first: MCQ

a. 6 hours
b. 24 hours
c. 48 hours
d. 1 week

3.N39 Adequate urine output during burn resuscitation is: MCQ

a. 0.1 mL/kg/hour
b. 0.5–1 mL/kg/hour
c. 3 mL/kg/hour
d. 100 mL/hour regardless of weight

3.N40 In burn patients, parenteral nutrition is preferred over enteral nutrition. TRUE / FALSE

a. True
b. False

3.N41 The three commonly used topical agents for burns are all EXCEPT: MCQ

a. Povidone iodine
b. Mafenide acetate
c. Silver nitrate solution
d. Silver sulphadiazine

3.N42 The ideal topical burn cream should be all EXCEPT: MCQ

a. Water-soluble base
b. Bactericidal
c. Toxic to viable cells
d. Not painful

3.N43 The exposure method of burn wound care is particularly indicated for burns of: MCQ

a. Feet
b. Circumferential limbs
c. Hands
d. Face, neck and perineum

3.N44 Advantages of the exposure method include all EXCEPT: MCQ

a. Avoids repeated dressing changes
b. No need for aseptic isolation
c. More comfortable
d. Dry air inhibits bacterial growth

3.N45 Partial-thickness skin grafts (Thiersch) are used to: MCQ

a. Cover deep face burns
b. Release eschars
c. Cover large raw areas produced by burns
d. Treat Curling's ulcer

3.N46 Full-thickness burns heal spontaneously within 2–3 weeks. TRUE / FALSE

a. True
b. False

3.N47 The most important prognostic factor in burns is: MCQ

a. Age
b. Site
c. Type
d. Extent

3.N48 The worst site of burns is the: MCQ

a. Hand
b. Perineum
c. Back
d. Face

3.N49 The major killer of burn victims is: MCQ

a. Contractures
b. Septicemia
c. Hypovolemia
d. Curling's ulcer

3.N50 The worst type of burn is: MCQ

a. High-voltage electric burn
b. Scald
c. Sunburn
d. Flame

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Skin functions	Barrier to microbes, prevents water loss, temperature regulation, sensation, vitamin D.	**Types of burn**	Scald (children), flame, electric (worst), chemical (acid/alkali), inhalation.
Extent	Major > 30%; intermediate 15–30% adults (10–30% children); minor < 15% adults (< 10% children).	**1st degree**	Epidermis only; erythema (sunburn); heals quickly.
2nd degree	Epidermis + part of dermis; blisters, painful; heals ~3 weeks if no infection.	**3rd degree**	Full thickness; leathery, painless; needs grafts.
Rule of nines	Head 9 · each arm 9 · each leg 18 · front trunk 18 · back trunk 18 · perineum 1.	**First aid**	Secure airway, strong analgesic, cool tap water 15 min (no ice).
Parkland	4 mL × kg × %TBSA Ringer's lactate: ½ first 8 h, ¼ second 8 h, ¼ third 8 h; max 50% TBSA in formula.	**Monitoring**	Urine output 0.5–1 mL/kg/h, vital signs, CVP in critical cases; NPO first 48 h.
Systemic complications	Inhalation injury, neurogenic & hypovolemic shock, renal failure, Curling's ulcer, MOF.	**Local complications**	Infection (4–7 days), constricting eschar, airway edema, compartment syndrome, contractures, Marjolin's ulcer.
Escharotomy	Release of constricting circumferential eschar in limbs/chest.	**Topical agents**	Silver sulfadiazine, silver nitrate, mafenide acetate; exposure or occlusive method.
Prognosis	Extent most important; face burns worst site; septicemia major killer; extremes of age worse.		

LECTURE 4

Trauma

الإصابات (Trauma)

Part A · Past Exam Questions

أسئلة الدفعات السابقة

4.1 An injury caused by the collision of an object with the body in which the object enters the body is called: كويز سابق (جراحة طوارئ)

a. Closed injury
b. Penetrating trauma
c. Internal injury
d. Blunt trauma

4.2 Which of the following is the primary survey in trauma care? كويز سابق (جراحة طوارئ)

a. Head-to-toe assessment
b. (ABCDE) assessment
c. History taking
d. Administering pain relief

4.3 A patient with blunt trauma to the chest develops hypotension and jugular vein distention. What condition should the nurse suspect? كويز سابق (جراحة طوارئ)

a. Hemothorax
b. Pulmonary embolism
c. Pneumothorax
d. Cardiac tamponade

4.4 Which intervention is most appropriate for a patient with a tension pneumothorax? كويز سابق (جراحة طوارئ)

a. Needle decompression
b. Applying a chest binder
c. Nasal oxygen therapy
d. Mechanical ventilation

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

4.N1 Trauma is defined as damage to the body caused by: MCQ

a. Tumors
b. Exchange of energy beyond the body's resilience
c. Infection
d. Degeneration

4.N2 Trauma is the leading cause of death in ages: MCQ

a. Neonates
b. 1–44
c. 45–65
d. Over 65

4.N3 Trauma is the _____ leading cause of death for all ages: MCQ

a. 1st
b. 2nd
c. 3rd
d. 5th

4.N4 **In trauma assessment, the key principle is:** MCQ

a. Take full history first
b. Do CT first
c. Don't get distracted with ugly injuries; initial assessment determines how to proceed
d. Treat the ugliest injury first

4.N5 **Mortality in trauma is decreased when:** MCQ

a. CT is done for every patient
b. The right patient gets to the right hospital in the right time
c. Patients are transported without assessment
d. All patients go to the nearest clinic

4.N6 **Seat belt injuries include all EXCEPT:** MCQ

a. Small intestine injury
b. Ankle fracture
c. Clavicle fracture
d. Mesenteric injury

4.N7 **In acceleration injury, when the car stops suddenly:** MCQ

a. There is no injury
b. Only the legs are injured
c. The person continues moving forwards and the head strikes the car
d. The person stops with it

4.N8 **Firearm injuries by rifles are:** MCQ

a. High velocity injuries
b. Thermal injuries
c. Low velocity injuries
d. Blunt injuries

4.N9 **Kinetic energy formula is:** MCQ

a. $K = M/V$
b. $K = M + V$
c. $K = MV/2g$ ($MV^2/2g$)
d. $K = 2gMV$

4.N10 **In high velocity injuries, shattered fragments of bone act as:** MCQ

a. Secondary missiles
b. Primary missiles
c. Shock waves
d. Blunt trauma

4.N11 **A fracture of the clavicle is an example of:** MCQ

a. Major trauma
b. Penetrating trauma
c. Multiple trauma
d. Minor trauma

4.N12 **Traumatic amputation of a leg is an example of:** MCQ

a. Minor trauma
b. Multiple trauma
c. Major trauma
d. Thermal trauma

4.N13 Multiple trauma is most often the result of: MCQ

a. Stab wounds
b. Falls
c. Motor vehicle accidents
d. Burns

4.N14 The Revised Trauma Score uses all of the following EXCEPT: MCQ

a. Respiratory rate
b. Heart rate
c. Systolic blood pressure
d. Glasgow Coma Scale

4.N15 Each parameter of the Revised Trauma Score is coded from: MCQ

a. 0–2
b. 0–4
c. 1–5
d. 0–10

4.N16 An RTS of less than _____ indicates the need for transport to a trauma center: MCQ

a. 8
b. 9
c. 11
d. 12

4.N17 The triage system sorts patients by: MCQ

a. Age
b. Arrival time
c. Colored labels according to injury and resources availability
d. Sex

4.N18 In the triage system of the lecture, the green label means the patient: MCQ

a. Will die anyway
b. Survives only with medical attention
c. Needs immediate surgery
d. Will survive anyway

4.N19 In the triage system of the lecture, the yellow label means the patient: MCQ

a. Is dead
b. Will survive anyway
c. Will survive only if receiving medical attention
d. Will die anyway

4.N20 ATLS is a protocol for management of: MCQ

a. Burns
b. Cardiac arrest
c. Major trauma
d. Poisoning

4.N21 The correct order of the primary survey is: MCQ

a. ABCDE
b. CABDE
c. DCBAE
d. AMPLE

4.N22 In the primary survey, 'A' stands for: MCQ

a. Analgesia
b. Airway with cervical spine protection
c. Assessment
d. AMPLE history

4.N23 The first question to assess airway patency is: MCQ

a. Did you eat?
b. Is there a fracture?
c. What is your name / can the patient talk?
d. What is the BP?

4.N24 Airway interventions include all EXCEPT: MCQ

a. Chin lift/jaw thrust
b. Suction
c. Needle decompression
d. Oral or nasal airways

4.N25 Cervical spine injury should be suspected in all EXCEPT: MCQ

a. Injury above the clavicle
b. Disturbed conscious level
c. Isolated finger injury
d. Multisystem trauma

4.N26 Cervical spine is immobilized by: MCQ

a. Sandbags only
b. Rigid collar and back board or manual in-line immobilization
c. Neck flexion
d. Soft pillow

4.N27 Airway patency ensures adequate ventilation. TRUE / FALSE

a. True
b. False

4.N28 Popcorn chest refers to: MCQ

a. Flail chest
b. Crepitus (surgical emphysema)
c. Tracheal deviation
d. Sucking wound

4.N29 Tension pneumothorax is treated immediately by: MCQ

a. Needle decompression
b. Intubation
c. Occlusive dressing
d. Chest X-ray first

4.N30 A sucking chest wound is treated by: MCQ

a. Occlusive dressing
b. Leaving it open
c. Needle decompression
d. Chest tube only

4.N31 Hemothorax is treated by: MCQ

a. Chest tube
b. Oxygen only
c. Occlusive dressing
d. Needle decompression

4.N32 Rapid assessment of hemodynamic status includes all EXCEPT: MCQ

a. Pulses
b. Level of consciousness
c. Skin color
d. Blood sugar

4.N33 Shock should be considered in every trauma patient. TRUE / FALSE

a. True **b.** False

4.N34 Cardiac tamponade causes _____ shock: MCQ

a. Hypovolemic **b.** Obstructive
c. Distributive **d.** Cardiogenic

4.N35 Distributive shock is due to: MCQ

a. Disruption of vasomotor tone **b.** Loss of blood
c. Pump failure **d.** Obstruction

4.N36 Circulation management in trauma includes all EXCEPT: MCQ

a. 1 litre Ringer's lactate or blood **b.** Oral fluids
c. Two peripheral IV lines **d.** Direct pressure on bleeding

4.N37 Target urine output in trauma resuscitation is: MCQ

a. 0.5–1 mL/kg/hour **b.** 10 mL/hour
c. No need to monitor **d.** 2–3 mL/kg/hour

4.N38 AVPU is used in: MCQ

a. Circulation **b.** Exposure
c. Airway assessment **d.** Disability (brief neurological exam)

4.N39 In AVPU, 'P' means: MCQ

a. Pressure **b.** Response to painful stimuli
c. Pulse **d.** Pupils

4.N40 Common causes of decreased conscious level in trauma include all EXCEPT: MCQ

a. Hypoxia **b.** Head injury
c. Hypertension **d.** Shock

4.N41 Decreasing mental status may be a sign of elevated intracranial pressure, managed by: MCQ

a. Trendelenburg
b. Fluids restriction only
c. Sitting patient up and hyperventilation
d. Lying flat

4.N42 In exposure, warm blankets are used to prevent: MCQ

a. Pain **b.** Infection
c. Hypothermia **d.** Shock

4.N43 AMPLE history includes all EXCEPT: MCQ

a. Medications **b.** Pulse rate
c. Last meal **d.** Allergies

4.N44 Rectal examination is part of the secondary survey. TRUE / FALSE

a. True **b.** False

4.N45 FAST is: MCQ

a. X-ray **b.** CT scan
c. Blood test **d.** Focused ultrasound for trauma

4.N46 Initial X-rays in trauma include all EXCEPT: MCQ

a. Chest **b.** Pelvis
c. Skull **d.** Cervical spine

4.N47 Hemothorax with more than _____ of blood initially indicates going to the OR: MCQ

a. 500 cc **b.** 1000 cc
c. 1500 cc **d.** 3000 cc

4.N48 Unstable patients with blunt or penetrating abdominal trauma should go to: MCQ

a. Discharge **b.** The OR
c. CT scan first **d.** Ward observation

4.N49 Stable patients with minor or no injuries are: MCQ

a. Admitted **b.** Taken to OR
c. Sent to ICU **d.** Discharged

4.N50 Once a life-threatening injury is discovered, intervention should be delayed until the secondary survey is complete. TRUE / FALSE

a. True **b.** False

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Trauma	Damage from energy exchange beyond the body's resilience with local and systemic effects.	**Epidemiology**	Leading cause of death age 1–44; 3rd for all ages.
Trauma system	Right patient, right hospital, right time.	**Blunt vs penetrating**	Blunt: no skin penetration (RTA, falls). Penetrating: object enters body (stab, gunshot).
Kinetic energy	KE = $MV^2/2g$ — velocity matters most (high-velocity firearms).	**RTS**	GCS + SBP + RR, each 0–4, total 0–12.
ATLS	Primary survey + resuscitation → secondary survey → definitive care.	**A**	Airway with C-spine protection: jaw thrust, suction, airway adjuncts, intubation.

B	100% O_2; needle decompression (tension pneumothorax); chest tube; occlusive dressing.
C	Direct pressure, two large-bore IVs, bloods, 1 L Ringer's or blood; UOP 0.5–1 mL/kg/h.
Shock types	Hypovolemic, cardiogenic, obstructive (tamponade, tension PTX), distributive.
D	AVPU (GCS later); causes: head injury, hypoxia, shock, drugs.
E	Undress fully, inspect back, warm blankets; Foley, Ryle tube, ECG, ABG.
Secondary survey	AMPLE history, head-to-toe exam incl. rectal, reassessment, X-rays, labs, FAST, CT.
To OR	Unstable chest/abdominal trauma, hemothorax > 1500 mL initially, surgical injury on imaging.

Answer Key — All Lectures

الإجابات

1.1. a — 1st part of duodenum
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.N1. b — Acute abdominal pain
Slide 2.

1.N2. a — Immediate laparotomy for all patients
Slide 2: history + examination + investigations.

1.N3. a — Colonic diverticulitis
Diverticulitis is a LEFT lower abdominal cause (slides 4–5).

1.N4. a — True
Listed under upper abdominal and medical causes.

1.N5. a — Mesenteric adenitis
RLQ: appendicitis, mesenteric adenitis, right ureteric colic, right ovarian cyst rupture/torsion.

1.N6. c — Left lower abdominal pain

1.N7. d — Acute appendicitis
Medical causes: gastroenteritis, FMF, uremia, MI, DKA.

1.N8. a — True

1.N9. c — Acute appendicitis
Slide 8.

1.N10. a — The second and third decades

1.N11. b — Wide-mouthed and well drained
In the elderly it is rare because the lumen is almost obliterated.

1.N12. d — 2/3 of cases

1.N13. a — Faecolith
Slide 10.

1.N14. b — False
Non-obstructive (catarrhal) is LESS likely to perforate; obstructive tends to be suppurative/gangrenous.

1.N15. b — 3–5 days
Slide 11 (course).

1.N16. b — Thrombophlebitis of the portal venous system with hepatic abscess

1.N17. a — Malignant transformation

1.N18. b — Alvarado score (MANTRELS)

1.N19. d — Tenderness in the right lower quadrant and leucocytosis
Total score = 10.

1.N20. b — False
Pain ALWAYS occurs before vomiting.

1.N21. b — Starts around the umbilicus then shifts to the right lower abdomen
Migrating pain.

1.N22. b — False
Absence of leucocytosis does NOT exclude the diagnosis.

1.N23. a — Deep palpation of the left lower quadrant

1.N24. c — Extension of the right thigh with the patient lying on the left side

1.N25. b — Perforation

1.N26. c — Little tenderness, late diagnosis and low immunity
Slide 18.

1.N27. d — Pain displaced upwards
Pain displaced upwards is a feature in PREGNANCY.

1.N28. b — Pyelitis
High incidence of abortion or premature labour.

1.N29. b — Tenderness that shifts with change of position, usually in children

1.N30. c — After exploration

1.N31. d — Clinical

1.N32. a — Young healthy adults with appendicular mass
Appendicular mass → conservative (Ochsner–Sherren).

1.N33. a — Ochsner–Sherren regimen then interval appendectomy after 3 months

1.N34. c — Sonar-guided aspiration, surgical drainage if failed
Abscess: persistent pain, fever, tachycardia, increasing mass; US confirms.

1.N35. c — 98%

1.N36. b — Obstruction of the cystic duct or neck of the gallbladder by a stone

1.N37. b — More than 6 hours
Initially diffuse colicky upper abdominal, then right hypochondrium, may radiate to the right shoulder.

1.N38. a — Acute cholecystitis

1.N39. d — Normal

1.N40. a — Air under the diaphragm
Air under diaphragm = perforated viscus.

1.N41. a — 3 days
Or conservative then cholecystectomy after 6 weeks (to allow adhesions to subside).

1.N42. c — Surgery: cholecystectomy or cholecystostomy

1.N43. a — NSAIDs or corticosteroids

1.N44. b — 6:1

1.N45. c — Perforated peptic ulcer

1.N46. d — Erect chest X-ray showing air under the diaphragm
Gastrografin meal shows leakage of contrast.

1.N47. b — Graham's omental patch

1.N48. b — Lack of fibre leading to chronic constipation and increased intracolonic pressure

1.N49. c — Sigmoid colon

1.N50. b — False
CT with oral and IV contrast; barium enema is NOT done in the acute attack.

1.N51. a — Exploration with Hartmann's procedure
Pericolic abscess → U/S guided drainage.

1.N52. c — Strangulation intestinal obstruction

1.N53. b — Atrial fibrillation or mural thrombus after MI

1.N54. d — Maximum symptoms with minimal signs
Until peritonitis occurs.

1.N55. d — CT angiography

1.N56. b — 24 hours
Viable bowel: embolectomy (embolism), bypass graft (thrombosis), anticoagulation (venous).

1.N57. b — Anticoagulation therapy

1.N58. c — Stones of the urinary tract
Slide 33.

1.N59. d — Air under the diaphragm

2.1. c — Bile duct stone
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.2. a — Conservative treatment
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.3. d — Serum lipase level
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.4. a — Indicated in all cases
إجابة مقترحة (المصدر مفيهوش إجابة رسمية). معظم الـ pseudocysts بتختفي لوحدها، والتصريف بيكون للكيس اللي فضل أكتر من 6 أسابيع أو كبير أو حصله infection.

2.N1. b — Upper abdominal retroperitoneal organ
Slide 2.

2.N2. b — 2nd part of duodenum

2.N3. b — 1–2 litres
Alkaline enzyme-rich juice (amylase, lipase).

2.N4. d — Bile duct stones
Bile duct stones 50%, alcohol 30%, idiopathic 20%.

2.N5. b — 30%

2.N6. c — Hypothyroidism

2.N7. a — True
Trauma: accidental, operative or during ERCP.

2.N8. b — Premature activation of pancreatic enzymes inside the gland leading to autodigestion

2.N9. d — SIRS

2.N10. c — Fatty acids binding calcium forming calcium soaps (fat necrosis)

2.N11. b — Epigastric pain radiating to the back, acute onset and gradually increasing

2.N12. d — Leaning forward (prayer position)

2.N13. a — Retroperitoneal

2.N14. a — Bruising around the umbilicus

2.N15. d — Bruising in the loin (flank)

2.N16. a — Pancreatic pseudocyst

2.N17. d — Pancreatic abscess
Pancreatic abscess and pseudocyst are LOCAL complications.

2.N18. a — Hypovolemia

2.N19. c — Hypocalcemia

2.N20. a — Acute appendicitis
DD: perforated PU, acute cholecystitis, MVO, leaking aortic aneurysm, MI.

2.N21. c — 1000 IU
Remains elevated for 2–3 days.

2.N22. b — False
Not specific — may rise in perforated peptic ulcer (usually < 500 IU).

2.N23. b — Lipase

2.N24. b — Detect patients who will need mechanical ventilation

2.N25. c — Hypoglycemia

2.N26. b — Gallbladder stones and dilated biliary tree

2.N27. b — CT scan with IV contrast

2.N28. a — CBD stones are suspected

2.N29. c — Conservative treatment (R regimen)

2.N30. b — Counteract spasm of the sphincter of Oddi

2.N31. a — Keeping the patient NPO
New concept: IV fluids until nausea and vomiting stop.

2.N32. a — Necrotizing pancreatitis (carbapenems preferred)

2.N33. c — ERCP

2.N34. b — Routine pancreatectomy for mild cases

2.N35. d — Inflammatory tissue (false cyst)

2.N36. b — 10% of cases
2–3 weeks after the attack.

2.N37. a — CT scan
Often painless and discovered on follow-up US.

2.N38. a — True

2.N39. c — 6 weeks
Internally into the stomach (cystogastrostomy) or jejunal loops.

3.1. d — 18% burns
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.2. a — Chemical burn
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.N1. b — Second (after muscles)

3.N2. b — Stratified squamous epithelium
Dermis = connective tissue.

3.N3. d — Lymph nodes

3.N4. a — True

3.N5. a — Formation of vitamin K

3.N6. c — Boiled liquids
Often affect children.

3.N7. d — Ignition of clothing

3.N8. c — Blood and bones are good conductors

3.N9. b — Hot gases
May affect upper or lower airway.

3.N10. d — Rule of 9
Modified in children (large head).

3.N11. c — 30%

3.N12. b — 15–30%
10–30% in children.

3.N13. b — 10%
< 15% in adults.

3.N14. b — 9%

3.N15. a — 1%

3.N16. c — Sunburn
Erythema only; heals rapidly.

3.N17. c — 3 weeks

3.N18. a — True

3.N19. a — Complete destruction of epidermis and dermis

3.N20. c — Hypertension as the main problem

3.N21. c — Pain

3.N22. b — Prolonged uncorrected hypovolemia

3.N23. a — Acute ulceration of the stomach and duodenum after burns
Rare now with antacids and H2 blockers.

3.N24. b — Days 4–7 postburn

3.N25. a — True
Prevention is by proper local wound care.

3.N26. a — Urgent escharotomy

3.N27. a — Urgent tracheostomy

3.N28. d — Malignant transformation in a long-standing burn scar

3.N29. b — Curling's ulcer
Curling's ulcer is SYSTEMIC.

3.N30. c — 15 minutes
Saline or tap water at room temperature.

3.N31. b — May induce more tissue damage

3.N32. a — True
< 15% adults, < 10% children: dressing + analgesia.

3.N33. d — First-degree sunburn of 5%
Also > 15% adults / > 10% children and burns with trauma or comorbidity.

3.N34. d — Check urine output

3.N35. b — 4 mL/kg/% burn lactated Ringer's per day

3.N36. c — The first 8 hours
1/4 in the second 8 hours, 1/4 in the third 8 hours.

3.N37. b — 50%
Otherwise serious over-infusion may occur.

3.N38. c — 48 hours

3.N39. b — 0.5–1 mL/kg/hour

3.N40. b — False
Enteral nutrition is preferred.

3.N41. a — Povidone iodine

3.N42. c — Toxic to viable cells

3.N43. d — Face, neck and perineum
And burns of one side of the trunk.

3.N44. b — No need for aseptic isolation
Exposure method requires isolation in a completely aseptic atmosphere.

3.N45. c — Cover large raw areas produced by burns
Full-thickness grafts for deep burns of face and hands.

3.N46. b — False
Partial-thickness heal in 2–3 weeks; full-thickness need skin grafts.

3.N47. d — Extent
Mortality ~50% if burn extent is 50%.

3.N48. d — Face
Associated inhalation injury.

3.N49. b — Septicemia

3.N50. a — High-voltage electric burn
Scalds with boiling water are the least harmful.

4.1. b — Penetrating trauma
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.2. b — (ABCDE) assessment
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.3. d — Cardiac tamponade
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.4. a — Needle decompression
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.N1. b — Exchange of energy beyond the body's resilience

4.N2. b — 1–44
3rd leading cause for all ages.

4.N3. c — 3rd

4.N4. c — Don't get distracted with ugly injuries; initial assessment determines how to proceed

4.N5. b — The right patient gets to the right hospital in the right time

4.N6. b — Ankle fracture

4.N7. c — The person continues moving forwards and the head strikes the car

4.N8. a — High velocity injuries
Knives and pistols are low velocity.

4.N9. c — K = MV/2g ($MV^2/2g$)
Velocity has the largest effect.

4.N10. a — Secondary missiles

4.N11. d — Minor trauma
Minor: single part/system, e.g. small 2nd-degree burn, cut requiring stitches.

4.N12. c — Major trauma
Serious single-system injury.

4.N13. c — Motor vehicle accidents

4.N14. b — Heart rate

4.N15. b — 0–4
Total RTS 0–12.

4.N16. c — 11

4.N17. c — Colored labels according to injury and resources availability

4.N18. d — Will survive anyway
Lecture: Red = die anyway, Yellow = survive only with medical attention, Green = survive anyway.

4.N19. c — Will survive only if receiving medical attention

4.N20. c — Major trauma
Primary survey + resuscitation → secondary survey → definitive treatment.

4.N21. a — ABCDE

4.N22. b — Airway with cervical spine protection

4.N23. c — What is your name / can the patient talk?

4.N24. c — Needle decompression
Needle decompression is a BREATHING intervention.

4.N25. c — Isolated finger injury
Also maxillofacial trauma.

4.N26. b — Rigid collar and back board or manual in-line immobilization

4.N27. b — False
Airway patency does NOT ensure adequate ventilation.

4.N28. b — Crepitus (surgical emphysema)

4.N29. a — Needle decompression

4.N30. a — Occlusive dressing

4.N31. a — Chest tube

4.N32. d — Blood sugar

4.N33. a — True

4.N34. b — Obstructive
Obstructive: physical obstruction reduces cardiac output.

4.N35. a — Disruption of vasomotor tone

4.N36. b — Oral fluids

4.N37. a — 0.5–1 mL/kg/hour

4.N38. d — Disability (brief neurological exam)
GCS is performed during the secondary survey.

4.N39. b — Response to painful stimuli

4.N40. c — Hypertension
Also alcohol or drug abuse.

4.N41. c — Sitting patient up and hyperventilation

4.N42. c — Hypothermia
Complete disrobing and inspect back.

4.N43. b — Pulse rate
Allergies, Medications, Past history, Last meal, Events.

4.N44. a — True
Physical exam head to toe including rectal exam.

4.N45. d — Focused ultrasound for trauma

4.N46. c — Skull
CT after resuscitation.

4.N47. c — 1500 cc

4.N48. b — The OR

4.N49. d — Discharged
Non-surgical high-risk injuries → admission.

4.N50. b — False
Intervention should NOT be delayed.